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Field Notes

Field Notes

- Based on CRAFT, Field Notes (FNs) are the “**core assessment activity**” for programmatic assessment and are a collaboration between the observer (eg.preceptor) and resident.
- Most often, this collaboration occurs in the form of feedback shared in response to a specific clinical encounter, focused on the resident’s performance and coaching involved, in the immediate context.
- Frameworks such as “domains of care”, “skill dimensions”, “CanMEDS roles” and “priority topics” help to ensure a good sample and diversity of feedback. For programmatic assessment, a good sample across these categories ensure comprehensiveness in feedback and coaching provided.

- Field notes are not designed to be ‘end-of-shift’ or ‘end-of-day’ summary assessments, but rather provision of feedback in response to specific encounters with coaching for improvement & refinement.
- Field notes are designed to be **low-stakes, formative assessments** on **day-to-day** clinical activities, and any individual field note assessment should not be taken as a formal, summative, decision-making basis. The overall portfolio of field notes can be used to provide insight into clinical exposures and overall entrustability for competencies.
- Currently, our program does not use a “minimum” number of field notes, but rather takes focus on the quality and sample of the Field Notes as they inform the acquisition of competencies over the training period. Generally, 1-2 Field Notes or more per week, done over the course of residency training, sampled across the Skills Dimensions and Domains of Care, would provide good evidence of feedback and learning and data points for guided reflection at periodic reviews. We encourage regular feedback and documentation, rather than completing them en masse prior to a competency committee meeting or periodic review.
- In cases where residents have identified gaps in their samples, or if a learning plan is developed to target specific competencies, quantitative minimums may be recommended in specific areas to ensure success.
- A good quality field note has the following attributes:
 - a) A response scale aligned to the reality map of the judges
 - b) A judgement rather than objective observation
 - c) A focus on competencies central to the activity observed
 - d) An assessor who is best placed to judge.
- Field notes are done on the T-Res platform and can be **preceptor-initiated** or **resident-initiated**:
 1. **Preceptor-initiated:** A preceptor who has a T-Res account can initiate a field note for any learner in the UBC Family Practice Residency Program. This is the most preferred way of documenting the feedback and discussion, and allows for a judgement based on the performance observed specific to the encounter.
 2. **Resident-initiated:** We understand that preceptors may not necessarily have ready access to T-Res, or may not be able to enter in comments immediately after an encounter amongst a busy clinical day. Residents are also able and encouraged to initiate a field note based on the feedback they received verbally through initiating their

own note. When tagged to a preceptor (who has a T-Res account), preceptors can endorse field notes and add additional comments; when this is done, the field note is referred to as a collaborative field note. Residents will not be penalized for resident-initiated field notes that are not endorsed by their preceptors.

- **For access to a T-Res account**, please contact your site's administrator. We encourage all primary preceptors of our residents to have access to T-Res for ideal provision and documentation of this feedback